RIVERSIDE REGIONAL MEDICAL CENTER

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PATIENT NAME: John Doe **DOB: 01/15/1984 (Age: 42)** **SEX: Male** **MRN: #RMC-984210**
ENCOUNTER DATE: 10/12/2026 **PLAN: United Healthcare** **STATUS: Outpatient** **PROVIDER: Dr. Sarah Jenkins, MD**
REQUESTED PROCEDURE: CT Scan of Head with Contrast (CPT 70460) | ICD-10: G44.89 (Atypical Cephalalgia)

CHIEF COMPLAINT:

Severe left-sided throbbing headaches persisting for 12 weeks with intermittent blurred vision and focal dizziness.

HISTORY OF PRESENT ILLNESS (HPI):

42-year-old male presents with persistent daily headaches lasting 12 weeks. Pain is localized to the left temporo-parietal region, rated 7-8/10. Headaches are atypical, awakening the patient from sleep on multiple occasions with associated nausea and left-sided visual disturbances.

PRIOR CONSERVATIVE MANAGEMENT:

Trial of conservative treatment attempted including outpatient physical therapy, ergonomic adjustments, and scheduled oral NSAIDs without clinical improvement. Symptoms remain refractory to non-pharmacologic and medical conservative measures.

PHYSICAL & NEUROLOGICAL EXAMINATION:

Vitals: BP 124/82, HR 72, SpO2 99% RA. Neurological examination abnormal: Cranial nerves II-XII intact except for subtle left-sided pronator drift on outstretched motor testing. Deep tendon reflexes asymmetric (2+ on right, 3+ on left brachioradialis). Coordination: mild dysmetria on left finger-to-nose testing. Sensation intact.

CLINICAL IMPRESSION & ASSESSMENT:

1. Intractable atypical chronic cephalalgia with focal neurological findings.\n2. Rule out intracranial pathology, space-occupying lesion, or intracranial neoplasm.

ORDER & CLINICAL RATIONALE:

CT Scan of Head with Contrast (CPT 70460). Intravenous contrast clinically indicated for further diagnostic evaluation and evaluation of suspected mass given focal motor asymmetry.

Electronically Signed by: Dr. Sarah Jenkins, MD, Neurologist *Date: 10/12/2026 14:32 PST*

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